

Reasoning

Very few of the included trials specifically evaluated patients with nodular or conglobate acne. As a result, indirect evidence from studies on severe papulopustular acne was used, focusing on the percentage reduction in nodules (NO) and cysts (CY). When indirect evidence was applied, the strength of recommendation for the respective treatment options was downgraded.

Systemic isotretinoin demonstrates superior efficacy in treating severe nodular and conglobate acne compared to systemic antibiotics or topical therapies alone.

The expert group concluded that systemic isotretinoin provides the greatest clinical effectiveness for severe nodular/conglobate acne. This assessment is supported only partially by published evidence due to the limited number of clinical trials specifically in conglobate acne.

Experts agree that isotretinoin's safety profile is manageable with appropriate initiation and monitoring. The benefits—significant treatment response, improved quality of life, and prevention of scarring—outweigh the potential side effects.

General Considerations

Choice of Type of Topical Retinoid

Adapalene should be preferred over tretinoin and isotretinoin.

Choice of Type of Systemic Antibiotic

Doxycycline and **lymecycline** should be preferred over minocycline and tetracycline.

Considerations on Isotretinoin and Dosage

- For **severe papulopustular acne** or **moderate nodular acne**, a dosage of systemic isotretinoin of **0.3–0.5 mg/kg/day** is recommended.
- For **conglobate acne**, a dosage of **≥0.5 mg/kg/day** is recommended.
- The treatment duration should be **at least 6 months**.
- If response is insufficient, the treatment duration may be extended.

Maintenance Therapy

Summary of Therapeutic Recommendations for Maintenance Therapy by Acne Type (Post-Induction)

Recommendations are based on available evidence and expert consensus. The strength of recommendation reflects both evidence quality and expert agreement.

Maintenance therapy is recommended, particularly for patients with a "particular need for maintenance treatment" (e.g., history of relapse, severe prior disease).

The low strength of recommendation in many cases reflects the lack of high-quality evidence on optimal maintenance regimens, not the necessity of maintenance therapy itself.

	Comedonal Acne	Mild to Moderate Papulopustular Acne	Severe Papulopustular / Moderate Nodular Acne	Severe Nodular / Conglobate Acne
High strength of recommendation	–	–	–	–
Medium strength of recommendation	–	–	–	–
Low strength of recommendation	Azelaic Acid or Topical Retinoid ²	Azelaic Acid or BPO or Topical Retinoid ²	Adapalene + BPO (f.c.) ³ or Topical Retinoid ²	Adapalene + BPO (f.c.) ³ or Azelaic Acid or BPO ³ or Low Dose Systemic Isotretinoin (max. 0.3 mg/kg/day) or Topical Retinoid ²
Alternatives for females ¹	–	–	Continued Hormonal Anti-androgens ⁴ + Topical Treatment (excluding antibiotics)	Continued Hormonal Anti-androgens ⁴ + Topical Treatment (excluding antibiotics)

¹ Low strength of recommendation.

² Preference for **adapalene** over tretinoin/isotretinoin.

³ For persistent inflammatory lesions.

⁴ e.g., combined oral contraceptives, spironolactone.